Comparative Case Note – Taking of Evidence in Court
[Your name] · [Student number] · [Date] · Words: 1479

Proving Causation through Expert Evidence: A Comparative Case Note on *State of Minnesota v. Derek Chauvin*

This case note concerns *State of Minnesota v. Derek Chauvin*, tried before Judge Peter Cahill in the Hennepin County District Court, Minneapolis, in March and April 2021. Chauvin, a former Minneapolis police officer, was charged in connection with the death of George Floyd on 25 May 2020, after he knelt on Mr Floyd's neck and back for nine minutes and twenty-nine seconds during an arrest for the alleged use of a counterfeit twenty-dollar bill. This note is based on the broadcast recordings of the opening statements (29 March), the testimony of Dr Martin Tobin (8 April), Dr Andrew Baker (9 April) and Dr David Fowler (14 April), Dr Tobin's rebuttal (15 April) and the closing arguments (19 April). Given the length and notoriety of the trial, this note focuses on one manageable issue: how the causation of death was proven, and how this would differ in a continental, traditionally inquisitorial, system.

Chauvin faced three counts: second-degree unintentional murder, committed while perpetrating a third-degree assault; third-degree murder; and second-degree manslaughter. Causing Mr Floyd's death is an element of all three, and it was the most contested one. The jury instructions defined it broadly: the prosecution had to prove that Chauvin's acts were "a substantial causal factor" in the death, and "the fact that other causes contribute to the death does not relieve the Defendant of criminal liability", unless a "superseding cause" was the sole cause.[1] The defence therefore had to raise reasonable doubt by pointing to alternative causes, whereas the prosecution only needed to show that the restraint was a substantial one. In a continental system, the legal test of causation would not be explained to lay jurors through instructions; it would be applied by professional judges, who must show in a written judgment how each element of the offence was fulfilled.

The opening statements already presented two competing stories of causation. Prosecutor Jerry Blackwell told the jury that the pressure on the neck and back of a handcuffed man who offered no resistance deprived Mr Floyd of oxygen, and that his death was neither a heart attack nor a drug overdose. Defence counsel Eric Nelson, instead, stressed reasonable doubt and promised evidence that Mr Floyd died of a cardiac arrhythmia caused by heart disease, fentanyl, methamphetamine and adrenaline, "all of which acted to further compromise an already compromised heart". Opening statements are not evidence, but they frame how the jury receives it. In an adversarial trial, the parties tell the story to a jury that has seen no file.[2] In a continental system, the presiding judge would open the hearing and summarise the case from the dossier, which he or she would have read beforehand. This

makes the hearing more efficient, but the judge does not enter the courtroom as a *tabula rasa*.

The prosecution's central expert was Dr Martin Tobin, a pulmonologist. His testimony began with questions on his qualifications to establish credibility, and he stressed that he had volunteered without payment. Dr Tobin had reviewed the body-camera and bystander footage from different angles and explained, frame by frame, that Mr Floyd died from "a low level of oxygen" that damaged his brain and stopped his heart. He calculated a normal respiratory rate of 22 breaths per minute, which in his view excluded fentanyl-induced respiratory depression, and he answered the absence of autopsy findings by stating that low oxygen does not leave "a fingerprint" and that a "static force" would not cause bruising. This shows that the video did not speak for itself: its probative value depended on the interpretation of an expert chosen by one party. Even an unpaid expert is selected because his opinion supports the party's case. In a continental system, the expert would normally be appointed by the court, with questions formulated by the judge, and the report would be part of the file available to all parties. Court appointment reduces the risk of partisanship, although it concentrates great influence in a single expert whose opinion is rarely challenged with the same intensity.

Dr Andrew Baker, the Hennepin County Chief Medical Examiner who performed the autopsy, was also called by the prosecution. Notably, he chose not to watch the video before the autopsy to avoid "preconceived notions", a concern that mirrors the criticism of continental judges who read the file before trial. He certified the cause of death as "cardiopulmonary arrest complicating law enforcement subdual, restraint, and neck compression", with heart disease, fentanyl and methamphetamine as other significant conditions, and the manner as homicide: the restraint was "just more than Mr Floyd could take by virtue of those heart conditions". Yet he found no physical sign of oxygen deprivation in the brain, and under cross-examination he accepted that the knee position would not anatomically cut off the airway and that, in other circumstances, the fentanyl level could be considered fatal. Cross-examination thus revealed that the State's own witness was useful to both sides. This raises the question whether such testing is a strength, since it exposes weaknesses that a written report might hide, or a risk, since lay jurors may be confused by technical concessions. In a continental hearing, the judge would question the expert first and the parties afterwards, and could exclude irrelevant questions, which gives more control but less adversarial pressure on the evidence.

The defence called Dr David Fowler, a forensic pathologist and former chief medical examiner. He testified that Mr Floyd died of "a sudden cardiac arrhythmia due to his atherosclerotic and hypertensive heart disease" during the restraint, with drugs, an adrenaline-secreting tumour and possible carbon monoxide from the police car as

contributing factors, and that the manner of death should be “undetermined”. He also referred to a guide on manner-of-death classification which states that a homicide label may reduce “the public perception that a ‘cover up’ is being perpetrated”, suggesting that public pressure can shape expert conclusions in a highly mediatised case. The prosecution responded with a sharp cross-examination and by recalling Dr Tobin, who rejected the carbon monoxide theory on the basis of the arterial blood gas. The result was a “battle of experts” in which twelve lay jurors had to choose between qualified physicians who contradicted each other. A continental court facing conflicting opinions could order a new expertise on its own initiative and would have to justify in writing why it preferred one opinion over another. Finally, Chauvin invoked his Fifth Amendment privilege and did not testify. In a continental trial, the defendant also has the right to remain silent but is normally invited by the judge to respond to the evidence, which may create psychological pressure to speak.

In closing, the defence reminded the jury of the presumption of innocence and of proof beyond a reasonable doubt, the highest standard of proof, while the prosecution argued that the 9 minutes and 29 seconds of pressure were a substantial causal factor and that Dr Tobin explained what happened, whereas the defence experts only explained what did not. On 20 April 2021, the jury returned unanimous guilty verdicts on all three counts without giving reasons. This is the most important contrast with continental systems, where professional judges must give a reasoned judgment explaining how the evidence proves each element. The adversarial model relies on a contest between the parties before a passive decision-maker, whereas the continental model relies on official inquiry and on accountability through reasons.[3] Here, the unreasoned verdict leaves unknown whether the jury accepted Dr Tobin’s asphyxia theory, Dr Baker’s cumulative explanation or simply the video.

Overall, the Chauvin trial shows the strengths of adversarial evidence-taking: expert opinions were tested thoroughly and in public, and weaknesses on both sides were exposed. However, the final weighing of complex medical evidence was left to lay jurors who gave no reasons. A continental approach would have offered a more neutral expert and a reasoned decision, at the cost of less adversarial scrutiny. Neither model is clearly superior; each balances truth-finding and fairness differently.

Notes

1. *State of Minnesota v. Derek Michael Chauvin*, No. 27-CR-20-12646 (Hennepin County District Court), Final Jury Instructions, 19 April 2021.
2. M. Damaška, ‘Evidentiary Barriers to Conviction and Two Models of Criminal Procedure: A Comparative Study’ (1973) 121 *University of Pennsylvania Law Review* 506.
3. M. Damaška, *The Faces of Justice and State Authority* (Yale University Press 1986).

Bibliography

M. Damaška, ‘Evidentiary Barriers to Conviction and Two Models of Criminal Procedure: A Comparative Study’ (1973) 121 *University of Pennsylvania Law Review* 506.

M. Damaška, *The Faces of Justice and State Authority* (Yale University Press 1986).

State of Minnesota v. Derek Michael Chauvin, No. 27-CR-20-12646, Final Jury Instructions, 19 April 2021.

State of Minnesota v. Derek Michael Chauvin, trial recordings, 29 March, 8, 9, 14, 15 and 19 April 2021 (Court TV / YouTube).